

Transmission-Based (Isolation) Precautions

Policy Number	POL-IC-024	Owner / Department	Infection Prevention & Control
Effective Date	2023-05-01	Version	4.1
Last Revised	2026-01-15	Next Review	2028-01-15
Approved By	Infection Prevention & Control Committee	Applies To	All clinical staff, providers, environmental services, and visitors in every inpatient and outpatient area

1. Purpose

To prevent transmission of infectious agents by applying Standard Precautions to all patients and adding Transmission-Based Precautions when a pathogen's route of spread requires it, consistent with the CDC Guideline for Isolation Precautions.

2. Definitions

- **Standard Precautions** — the baseline practices applied to all patients regardless of diagnosis: hand hygiene, PPE based on anticipated exposure, respiratory hygiene, and safe injection practices.
- **Transmission-Based Precautions** — Contact, Droplet, and Airborne precautions added when a pathogen is known or suspected.
- **AIIR** — airborne infection isolation room: a single room at negative pressure with ≥ 12 air changes per hour, air exhausted outside or HEPA-filtered.
- **Fit-tested respirator** — an N95 or higher respirator required for airborne precautions.

3. Precaution Types

Initiate precautions empirically on clinical suspicion — do not wait for confirmatory testing. Combine precaution types when a pathogen spreads by more than one route.

Type	PPE / placement	Example pathogens
Contact	Gown and gloves on room entry; single room preferred; dedicated equipment.	MRSA, VRE, <i>C. difficile</i> , RSV, scabies.
Droplet	Surgical mask within 6 feet; single room preferred; patient masked for transport.	Influenza, pertussis, <i>N. meningitidis</i> , mumps.
Airborne	Fit-tested N95 or PAPR; AIIR with door closed; patient masked for transport.	<i>M. tuberculosis</i> , measles, varicella, disseminated zoster.

IMPORTANT — A patient with known or suspected airborne disease (e.g., active TB, measles, or varicella) must be placed immediately in an AIIR with the door closed, and staff must wear a fit-tested N95 or PAPR before entry. If no AIIR is available, mask the patient, move them to a private room, and contact Infection Prevention at once.

4. Severity & Reporting

Isolation events are classified under POL-RM-013. A failure to isolate that results in an exposure of staff or other patients to a high-consequence airborne pathogen (e.g., measles or active pulmonary TB) is escalated as SEV-1 and requires notification of the Nursing Supervisor, Risk Management, Infection Prevention, Employee Health, and the Administrator-on-Call immediately and within 1 hour. A precaution breach causing a lower-risk exposure needing intervention is SEV-2 (notify within 4 hours). A breach with no exposure, or one intercepted at the door, is SEV-3 or a Near Miss, reported within 24 hours.

5. Documentation Requirements

- Precaution type initiated, the indication, and date/time started and discontinued.
- Room type used (single room, AIIR) and signage posted.
- For any breach or exposure: persons exposed, the event report, assigned severity, and all notifications; exposure follow-up coordinated with Employee Health.

6. References

- CDC/HICPAC, Guideline for Isolation Precautions: Preventing Transmission of Infectious Agents in Healthcare Settings (2007, with updates).
- The Joint Commission, Infection Prevention and Control (IC) standards; CMS Conditions of Participation, 42 CFR 482.42.
- Related: POL-RM-013 (severity & reporting), POL-IC-020 (hand hygiene), POL-IC-026 (tuberculosis).